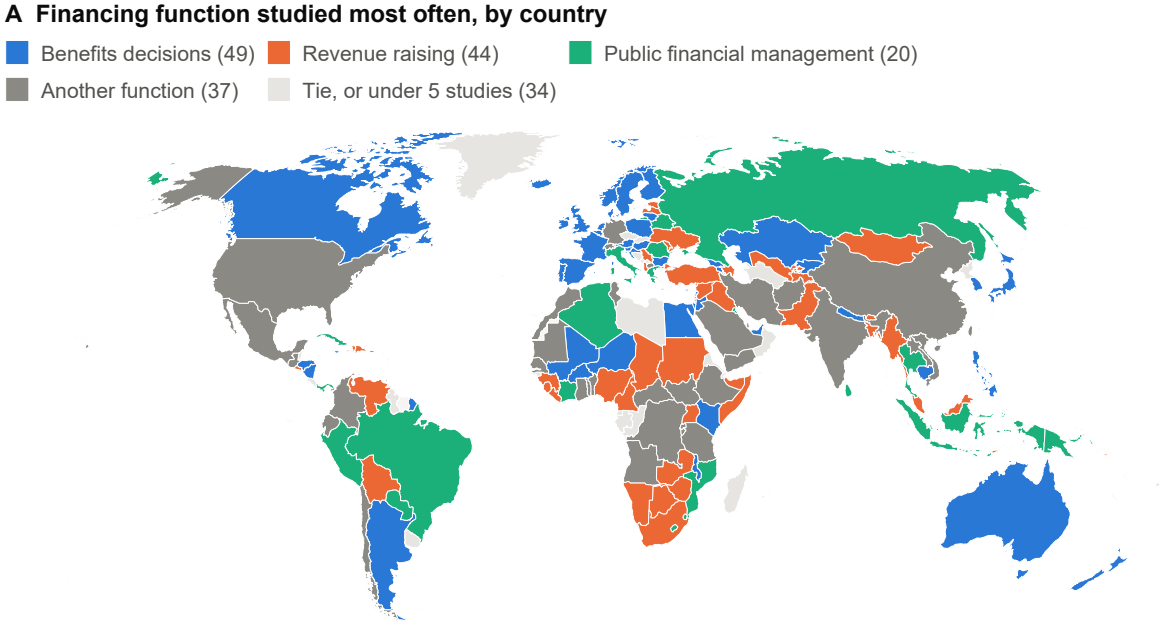
(Un)equal weight: which health financing functions research focuses on

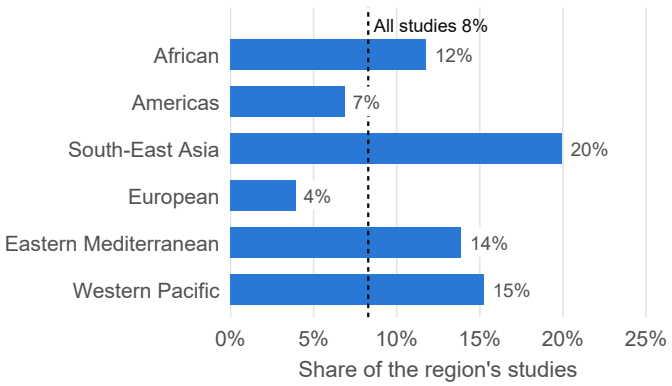
A Financing function studied most often, by country



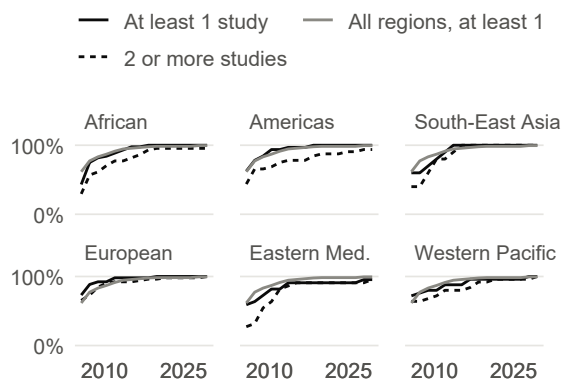
B Financing functions studied, by WHO region

	African (n = 4,133)	Americas (n = 19,462)	South-East Asia (n = 2,400)	European (n = 9,785)	Eastern Mediterranean (n = 1,751)	Western Pacific (n = 7,862)
Benefits decisions	**22%**	25%	19%	**29%**	20%	**27%**
Purchasing	15%	26%	12%	26%	19%	22%
Pooling	20%	**28%**	**24%**	10%	16%	22%
Public financial management	19%	12%	21%	19%	15%	13%
Revenue raising	22%	10%	21%	13%	**22%**	14%
Financing system as a whole	14%	6%	13%	13%	20%	11%
Supply-chain financing	15%	5%	12%	8%	14%	9%
Capital investment: infrastructure	5%	3%	5%	5%	5%	4%
Capital investment: other inputs	6%	3%	6%	3%	6%	4%

C Financial protection as an outcome



D Countries with a study, over time



HFF dataset v2.5.1: 53,411 studies, 47,693 study-country pairs. A: each country takes the financing function tagged most often among the studies naming it (numbers in brackets are countries; 'Under 5 studies' = fewer than 5 function-tagged studies; 'Another function' = pooling, the financing system as a whole, purchasing or supply-chain financing). B: share of each region's function-tagged studies that address each function, bold = the function studied most; a study can address several functions, so columns sum to more than 100%. C: share of the region's studies with financial protection among their outcome domains. D: cumulative share of the region's countries named by at least one, or two or more, studies, 2010-2025 (2026, a partial year, is left out). Regions: WHO; Taiwan and Hong Kong (Western Pacific) and Kosovo (European), which WHO does not assign, take the WHO region of their UN M49 sub-region. Studies naming countries in several regions count in each. 'Other' and 'Unclear' function tags are left out throughout.